



COMPREHENSIVE RESIDENTIAL STAFF BINDER

HR · Payroll · Attendance · Health & Safety · Youth Care Practice · Documentation · Medication

All Residential Care Programs — Children and Youth

Document Reference	ECC-STAFF-BND-2026-001
Version / Effective Date	1.0 — July 2026
Applies To	All residential program staff: Child & Youth Care Workers, Shift Leads, casuals, and students/volunteers where indicated
Approved By	Executive Director
Review Date	July 1, 2027 (annual)
Governing Documents	This binder summarizes and applies: ECC-HR-POL-2026-001 (HR Policy Binder v3); ECC-MED-2026-001 (Medication Administration Manual); ECC-OHS-POL-2026-002 (OHS Binder); ECC-BSP-2026-001 (Behaviour Support Manual); ECC-RC-POL-2026-003 (RC Program Manual v4); MCFD SHSS Standards (rev. April 2026); BC Employment Standards Act. Where this summary and a governing document differ, the governing document prevails.

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PART 1 — EMPLOYMENT & HR POLICIES

1.1 Hiring, Screening, and Probation

Screening	Every staff member completes, BEFORE any unsupervised contact with youth: Criminal Record Check (CRRRA — renewed every 5 years), interview, personal and employment references, and a medical reference (SHSS G.1.7)
Ongoing obligation	You must report to the PM immediately if you are charged with or convicted of a criminal offence, placed on parole/probation, or develop any condition (including substance use) that could impair your ability to care for youth (SHSS C.2.9)
Probation	New staff serve a 3-month probationary period with a documented probation review; medication authorization is issued only after training and competency demonstration (see Part 7)
Job description	You receive a current written job description at hire; duties include your responsibilities under each youth's care plan (SHSS G.1.9)
Personnel file	Your file holds: CRC, references, qualifications, orientation acknowledgement, first aid certificate, evaluations, and any discipline; you may request to view your file through the PM

1.2 Code of Conduct and Ethics

Professional boundaries	Relationships with youth are professional and therapeutic at all times; no romantic, sexual, financial, or secretive relationships with any youth or former youth in care; no babysitting or private arrangements with youth or their families without ED approval
Gifts and money	No borrowing from, lending to, buying from, or selling to youth; gifts only per program practice and documented; no acceptance of significant gifts from families or service providers
Conflict of interest	Disclose to the PM any relationship or interest that could conflict with your duties (SHSS H.1.10)
Respectful workplace	ECC maintains a harassment-free workplace; bullying, harassment, and discrimination are not tolerated toward youth, colleagues, or families; complaints go to the PM (or ED if the PM is involved) and are handled per the grievance procedure without reprisal (SHSS G.4.7)
Substance use	No alcohol, cannabis, or non-prescribed impairing substances during shifts or on program premises; report impairing prescription effects to the PM; arriving impaired is grounds for immediate removal from shift and discipline
Social media & devices	No youth names, images, or identifying information on personal devices or any social media, ever; no photographing youth or records except on program devices per policy with SW written permission for any public use (SHSS B.3)

1.3 Confidentiality and Privacy

Standing duty	All youth information is confidential during and after employment; share only with those with a professional need to know; sign the confidentiality agreement at orientation
Youth privacy	Respect private space and belongings; visits with family and legal counsel are private; body and strip searches never occur (SHSS B.3)
Breach	Any suspected privacy breach is reported to the PM the same day

1.4 Performance, Supervision, and Discipline

Supervision	You receive regular supervision (ECC standard: monthly) covering your responses to youth, methods, safety, and development (SHSS G.4)
Evaluation	Written performance evaluation at the end of probation and annually thereafter (SHSS G.4.5)
Progressive discipline	Coaching → verbal (documented) → written warning → suspension/final warning → termination; serious misconduct (abuse, prohibited practices, breach of trust, safety endangerment) may result in immediate termination and mandatory external reporting
Grievance	You may grieve any decision through the written grievance procedure in ECC-HR-POL-2026-001 without reprisal

PART 2 — PAYROLL & COMPENSATION

Payroll is administered per the BC Employment Standards Act (ESA). The figures below are ESA minimums summarized for staff information; your employment agreement may provide better terms. Confirm specifics with the PM or payroll.

2.1 Pay and Timekeeping

Pay periods	Staff are paid at least semi-monthly, and no later than 8 days after the end of the pay period (ESA)
Timesheets	Record actual hours worked each shift, including handover time; submit by the payroll deadline posted at the program; falsifying a timesheet is serious misconduct
Wage statements	You receive an itemized wage statement each payday showing hours, rate, overtime, and deductions
Deductions	Only lawful deductions are taken: statutory (tax, CPP, EI), court-ordered, or written assignments that benefit you (e.g., benefits premiums). ECC does not deduct pay as a penalty or fine, and does not deduct for business costs such as breakage, shortages, or damage (ESA s.21)
Minimum daily pay	If you report for a scheduled shift, you are paid for at least 2 hours even if the shift ends early (4 hours where the scheduled shift was over 8 hours), unless you are unfit to work or fail to meet conditions of employment (ESA)

2.2 Overtime, Statutory Holidays, Vacation, and Leaves

Overtime	ESA minimums: 1.5× after 8 hours in a day; 2× after 12 hours in a day; 1.5× after 40 hours in a week. Overtime must be pre-approved by the PC/PM except where required by an emergency or mandated coverage; record it on the timesheet
Statutory holidays	Eligible staff (employed 30+ days and worked/earned wages on 15 of the last 30 days) receive an average day's pay for the holiday; working the holiday: 1.5× for hours worked (2× after 12) plus the average day's pay
Vacation	ESA minimums: 2 weeks after 1 year (4% vacation pay); 3 weeks after 5 years (6%); request vacation in writing to the PC with as much notice as possible — coverage must be arranged before approval
Illness & injury leave	After 90 days of employment: 5 paid sick days plus 3 unpaid per year (ESA); ECC may request reasonably sufficient proof
Other job-protected leaves	Maternity/parental, family responsibility (5 days), compassionate care, bereavement, critical illness of a child, leave respecting domestic or sexual violence, and jury duty apply per the ESA; notify the PM as early as possible
Shift premiums / on-call	Any overnight, awake-night, or on-call premiums are per your employment agreement and the posted schedule policy

PART 3 — SCHEDULING, ATTENDANCE, AND DRESS CODE

3.1 Scheduling and Shift Handover

Ratios	Staffing ratios per the program model are maintained at all times (SHSS G.2); the outgoing shift does not leave until properly relieved
Handover	Arrive in time to receive verbal handover, read the shift log, complete the medication and CDS shift-change counts, and accept key transfer BEFORE your shift starts (ECC-MED §11)
Shift exchange	Shift swaps require PC approval in advance; both staff remain responsible until the swap is approved and posted
Working alone	Follow the working-alone/check-in procedure in the OHS binder; know the on-call schedule

3.2 Late Arrival Rules

If you will be late	Phone (not text) the Shift Lead or PC the moment a delay is known — at latest 30 minutes before shift start; state your expected arrival time
Pay	Wages are paid for time actually worked: late arrival is unpaid for the actual time missed per payroll rounding. Consistent with ESA s.21, no penalty or flat-rate fine (e.g., \$ per minute) is deducted — repeat lateness is addressed through discipline
Recording	Every late arrival is documented in the shift log and reported to the PC (time scheduled, time arrived, notice given, reason)
Discipline ladder	1st in 90 days: documented verbal coaching → 2nd: written warning → 3rd: PM meeting and formal warning → further: up to termination. No-notice lateness moves one step higher; 2+ hours with no contact is an unauthorized absence

3.3 Absenteeism Rules

Reporting	Phone the Shift Lead or PC as soon as the need is known — minimum 4 hours before shift start wherever possible; notice is effective when acknowledged, not when a message is left
Proof	Statutory sick leave applies; a medical note may be requested after 3 consecutive missed shifts or where a pattern emerges
No-show	Missing a shift without notice: PM meeting and written warning at minimum — youth safety depends on coverage
Job abandonment	3 consecutive scheduled shifts missed with no contact is treated as job abandonment
Medication continuity	If you are medication-authorized and absent, confirm the PC knows so administration is reassigned; a missed dose caused by unreported absence is investigated as a medication incident (ECC-MED §26)

3.4 Dress Code

Standard	Clean, professional-casual clothing in good repair; closed-toe, non-slip footwear at all times; clothing must allow safe physical activity, cooking, and emergency response
ECC-issued blazer	May be issued for community outings, school meetings, appointments, and professional events; ECC property; returned on separation
ECC-issued fanny pack	May be issued for shift use (gloves, personal alarm, program phone, keys); keys never left unattended; returned at end of shift/separation
Not permitted	Offensive or drug/alcohol-themed graphics; revealing clothing; heavy scents (scent-reduced workplace); unsafe jewelry

PART 4 — HEALTH & SAFETY

Your OHS rights & duties	You have the right to know hazards, participate in H&S, and refuse unsafe work per WorkSafeBC; report all hazards, injuries, and near-misses to the Shift Lead/PC the same shift; complete WorkSafeBC forms for any work injury
Fire safety	Know the fire/emergency plan posted on each floor; participate in the monthly fire drill and the drill after every new placement; know extinguisher locations (SHSS F.4.7)
First aid	Maintain a valid basic first aid certificate at all times (condition of employment, SHSS G.3.8); know the first aid kit location
Water safety	Check bath/shower temperature every use: 37–38°C, never above 42°C; safe beverage temperatures (SHSS C.1.6, April 2026)
Violence prevention	Follow the Behaviour Support Manual (ECC-BSP-2026-001) de-escalation practices; report all incidents of aggression; debrief after critical incidents
Hazardous products	Store cleaners and hazardous materials locked and inaccessible to youth (SHSS F.4.8); follow WHMIS labels and SDS
Infection control	Hand hygiene before food handling and every medication administration; gloves for bodily fluids and topical medications; follow illness-outbreak directions from the PC
Vehicles	Only licensed, approved drivers transport youth; certified restraints; children 12 and under in the back seat; report any vehicle defect before use (SHSS F.1)

PART 5 — YOUTH CARE PRACTICE STANDARDS

5.1 Youth Rights

Rights information	Every youth is told their CFCSA s.70 rights in an understandable way, including how to contact the Representative for Children and Youth; you support access to the SW, RCY, Ombudsperson, Helpline for Children, and licensing officer (SHSS B.1, C.1.7)
Complaints	Take every complaint seriously; no reprisal ever; help the youth use the complaint procedure; report complaints to the PC same day (SHSS B.2)
Dignity & identity	Care is respectful of culture, Indigenous identity and community, religion, and gender identity — gender-affirming care is mandatory practice (SHSS D.1)

5.2 Behaviour Support and Prohibited Practices

Positive practices only	Use the strategies in each youth's care plan and the Behaviour Support Manual; consequences are related, respectful, reasonable, and documented (SHSS D.7)
PROHIBITED — always	Physical discipline of any kind; deprivation of food, clothing, shelter, or sleep; degradation, humiliation, or verbal abuse; seclusion/locked confinement; coercion; arbitrary denial of family contact; using medication as a restraint or behaviour tool (ECC-MED §1.1). Any instruction to use a prohibited practice must be refused and reported to the PM/ED immediately
Physical restraint	Only to prevent immediate physical harm (or as director-approved in the care plan), only by trained staff, minimum force, shortest time; every restraint is a reportable incident with a debrief within 24 hours (SHSS C.3)

5.3 Reportable Incidents

Notify IMMEDIATELY	The youth's social worker is notified immediately (after hours: MCFD Centralized Screening 1-800-663-9122) for: death; injury/illness needing medical treatment; abuse or neglect allegations; self-harm or suicide gestures; missing youth; police involvement; physical restraint; prohibited practices; unauthorized removal; marked behaviour change; school suspension; unauthorized overnight absence; relocation; any other safety circumstance (SHSS C.2.8)
Then	Notify the PC same shift; written incident report within 24 hours; never delay the SW notification to finish paperwork
Duty to report abuse	Every person in BC must report a child protection concern directly under CFCSA s.14 — this duty is personal and cannot be delegated to a supervisor

5.4 Daily Care Duties

Daily log	Record significant aspects of each youth's day and any routine changes, every shift (SHSS C.1.11)
Wellness checks	Complete and document daily safety and wellness checks per each care plan (SHSS C.1.12)
Health & meals	Support timely medical/dental care; nutritious meals per Canada's Food Guide; report health concerns to the PC; serious illness/injury → PC immediately, SW and licensing ASAP (SHSS D.6)
Visitors	All visitors are screened and monitored; no visitor has access to medications or records (SHSS C.1.5; ECC-MED §10)

PART 6 — DOCUMENTATION STANDARDS

If it isn't documented, it didn't happen. Every record is a legal document reviewable by MCFD, licensing, and the courts.

Same shift	Daily logs, wellness checks, MAR entries (at administration — never pre-signed), incident reports, refusal records: completed before you leave
How to write	Black/blue ink only; legible; factual, objective, respectful language; record observations and actions, not opinions or labels; quote the youth where relevant
Corrections	Single line through the error, initial and date, write the correct entry; never erase, obscure, white-out, or remove any record
Late entries	Marked 'LATE ENTRY' with actual date/time of writing and the date/time of the event; reason noted
Signatures	Sign/initial every entry; expand initials in the signature legend where required; never sign for another person
Deadlines	Medication Error Report (App D) and Refusal Record (App H): before end of shift. Reportable incident written report: within 24 hours. CDS Count Sheet: at the count, both staff sign
Confidentiality	Records never leave the program; no photos of records; nothing on personal devices; secure storage; records belong to the director and are never destroyed (SHSS E.7)

PART 7 — MEDICATION ADMINISTRATION (STAFF SUMMARY)

This part summarizes ECC-MED-2026-001 for daily use. The full manual governs. **You may not administer ANY medication — prescribed or over-the-counter — until you hold current written program-specific authorization from the PM.**

7.1 Authorization and Training

Before any medication duty	Complete BC MALM (or equivalent); in-person modules with the PC; competency demonstration (Seven Rights, MAR, CDS count, error response); sign Appendix I; receive written authorization (Appendix A). Authorization is program-specific and expires after 12 months (ECC-MED §4)
EpiPen / Naloxone	Training at orientation; know their locations — the EpiPen is NOT locked in the cabinet (ECC-MED §23)

7.2 The Seven Rights — Every Dose, Every Time

1. Right Youth	Two identifiers — ask the youth to state their full name and confirm date of birth; never rely on appearance or room
2. Right Medication	Read the label three times: removing from storage, preparing, before administering; label must match the MAR exactly
3. Right Dose	Confirm/calculate before preparing; unusual dose → STOP and call pharmacy or prescriber
4. Right Route	Route matches label and MAR; routes are never interchanged
5. Right Time	Within 30 minutes before/after scheduled time; more than 30 minutes late → consult pharmacy before administering; never double-dose
6. Right Reason	Know why this youth takes this medication; unknown purpose → ask the PC before administering
7. Right Documentation	Initial the MAR immediately AFTER administering — never before, never at end of shift; pre-signing is a serious error

7.3 Core Rules

Storage & keys	Cabinet locked except during authorized administration; CDS double-locked; keys transferred and logged at shift change; lost key → PC immediately (ECC-MED §9–10)
CDS counts	Two authorized staff count every controlled substance at every shift change; two staff at every CDS administration (one gives, one witnesses); discrepancies investigated before the outgoing shift leaves (ECC-MED §11, §21.1)
MAR codes	Initials = given; R = refused; H = held on prescriber direction; NA = not available (notify PC); S = observed self-administration. Every scheduled dose gets an entry — no blanks
Refusal	Never force, coerce, bribe, or hide medication in food (covert administration is prohibited and reportable); explore the reason, inform, offer time; document (R + Appendix H); notify PC same shift (ECC-MED §25)
PRN medications	Only per the approved written protocol: assess the target symptom first; record effectiveness after (Appendix F); 3+ uses in 7 days is flagged to the PM and prescriber (ECC-MED §18)
OTC medications	Nothing over-the-counter — including vitamins and supplements — without documented authorization (ECC-MED §19)

7.4 If Something Goes Wrong

Any error or near-miss	1) Assess the youth. 2) 911 if any serious concern, overdose, or CDS involved. 3) BC Poison Control 1-800-567-8911 whenever uncertain. 4) PC/on-call within 15 minutes. 5) No further doses until directed. 6) Stay with the youth. 7) Appendix D before end of shift. Reporting is about safety, not blame — hiding an error is the misconduct (ECC-MED §29)
Anaphylaxis	EpiPen immediately (orange tip to outer thigh, hold 10 seconds) → 911 immediately → second dose after 5–15 minutes if no improvement → stay with the youth (ECC-MED §23.1)
Suspected opioid overdose	911 → Naloxone per kit → recovery position/rescue breathing → effects can return as Naloxone wears off, EMS assessment is mandatory (ECC-MED §23.3)
Adverse reaction	Severe (breathing difficulty, swelling, rash with fever, seizure, unresponsive): 911. Otherwise: PC immediately; no next dose without prescriber direction (ECC-MED §36)

PART 8 — TRAINING & CERTIFICATION REQUIREMENTS

Requirement	When	Renewal
Orientation + written acknowledgement (SHSS G.3.6–.7)	Before first shift	Re-acknowledged on policy updates
Basic first aid certificate (SHSS G.3.8)	Before duties	Before certificate expiry
Gender-affirming care training (SHSS G.3.2(b))	At orientation	Annually
MALM + medication authorization (ECC-MED §4)	Before any medication duty	Every 12 months
EpiPen / Naloxone training (ECC-MED §23)	At orientation	Annually
Behaviour support / de-escalation (ECC-BSP)	At orientation	Per program schedule
Annual competency review (Seven Rights, MAR, CDS, errors)	Annually with PC/PM	Before authorization expiry
Performance evaluation (SHSS G.4.5)	End of probation	Annually

PART 9 — STAFF ACKNOWLEDGEMENT

I acknowledge that I have received, read, and understood the Comprehensive Residential Staff Binder (ECC-STAFF-BND-2026-001), including the HR, payroll, attendance, dress code, health and safety, youth care practice, documentation, and medication administration requirements. I understand that the governing policy documents prevail over this summary, that prohibited practices must never be used, and that I may not administer any medication without current written authorization.

Staff Name (print)	
Position / Program	
Signature / Date	
Blazer issued (Y/N, date, size)	
Fanny pack issued (Y/N, date)	
First Aid expiry / MALM expiry	
Medication Authorization expiry	
PC Signature / Date	
PM Signature / Date	